

DIPLOMA IN SOCIAL SERVICE

SOCIAL SERVICE PROGRAMME DEVELOPMENT

SSC-PMT-5022-1.1

Reflective Practice

Assignment Cover Sheet

Name of Candidate	Host Organisation
Name of Assessor	Contact Details
Name of IA Supervisor	Date of Submission
CANDIDATE'S DECLARATION a. I, hereby declare that all the entries in this journal are my own effort and do not involve plagiarism or works of other people whose services I have engaged. b. I have not allowed, and will not allow anyone to copy any of my Written Assignments with the intention of passing it off as his/her own works. Signature: _____ Date: _____ <i>*Attach this Assignment Cover page to your RP journal.</i>	

Assessment Method 3: Reflective Journal

Instructions for Candidates

A reflective journal is a personal record of your learning experiences. It is a tool which enables you to record and reflect upon your observations and responses to situations, which can then be used later to explore and analyse ways of thinking and being in contexts. Journals, although generally written, can also contain images, drawings and other types of reference materials.

This Reflective Journal uses the basic framework for self-reflection which is *description, reflect and application*. For this Reflective Practice assignment, you are required to complete this reflective journal of experiences gathered during the industrial attachment.

Instructions

Pre-Attachment

1. You will be assigned to a Host Organisation (HO) for your industrial attachment (IA). A HO supervisor will be appointed to mentor you.
2. If you have any questions regarding the entries in the RPJ, email your assessor for clarifications.
3. HMTA office will contact you to book the date for the Final Assessment before the IA. The date and timeslot selected by you is final. Any changes may incur an administrative fee.

During Attachment

4. A week prior to your IA, craft some guiding questions related to the TSC listed in the journal. You may gather in groups of 2 or 3 to design the questions. These questions will help you to target the areas you would like to learn during the attachment.
5. Preferable to make an appointment with your IA-Supervisor within the first week of attachment to gather the information related to the TSC. In addition, reflect on your work roles (if any) performed each workday which are related to any of the TSC in the RP journal.
6. Document the information gathered from your IA-supervisor, your personal observations or the work roles you performed every day in Column 2 of the

RPJ for the respective modules.

7. The writing of the RPJ is an individual and personal assignment.

8. The RPJ should be used to explore situations from a personal perspective, but generally within the context of learning from your experiences. Write your personal reflections in Column 3 on the information gathered in Column 2 of the RPJ. To guide you in writing your entry, you may find the following sample questions useful:

- *Is there a framework?*
(a) If no, why not? What would you have recommended to the HO?
(b) If yes, does it serve its intended purpose? Is there a set of principles to guide the organisation in managing the framework? What would you have recommended to the HO?
- *What were the incidents? What were the outcomes? Were these incidents preventable? If yes, what could be done differently to prevent the incidents?*

9. You are advised to make your entries after each workday and at your own pace.

Post-Attachment

10. Email the RPJ to your assessor 5 days after the end of the industrial attachment and cc to the Course Co-ordinator.

During RP Presentation

11. You have 30 mins to present your entries for each module to the assessor. The following sequence is recommended:

- Provide a brief overview of the information gathered.
- Share your thoughts and feelings on what you have learnt.
- Highlight your key learning points from your experience.

12. Assessor may pose a few questions to clarify your entries.

13. Candidate must demonstrate competency in both the RPJ and the RP presentation, in order to be deemed competent for this RP.

Quality of the Journal Entries

Entries should:

- be clear and reflect your ability to think critically and objectively.
- be in complete sentences. You are allowed to number or bullet your entries for easy reading.
- demonstrate you have acquired the respective technical skills and competencies of this module.
- journal all interactions, experiences, thoughts and relevant information which are deemed relevant
- not breach client confidentiality

avoid using names when referring to people at the workplace

Reflective Journal Criteria

Performance Criteria	Self-Reflection Journal Prompts	Student Reflection Input
LU2: Reframe programme planning and development <u>Abilities (A5)</u> Determine senior leadership and multi-disciplinary team support for programme planning and development in the community and social service settings.	1. Learning points on AAC or day Care Centre meetings such as debriefs, roll-calls, multi-discipline team meetings, meetings with your team mates (aka classmates), meetings with volunteers, etc. 2. Reflect on meeting details e.g. agenda items and what other important factors to take note of?	
LU4: Devise strategic plan and framework in programme evaluation <u>Abilities (A4)</u> Plan strategic directions in programme evaluations and methods	3. Reflect and document in detail the use of a planning triangle to plan and evaluate your participated or initiated program or activity.	

LU2: Reframe programme planning and development

Abilities (A5)

Determine senior leadership and multi-disciplinary team support for programme planning and development in the community and social service settings.

1. Learning points on AAC or day Care Centre meetings such as debriefs, roll-calls, multi-discipline team meetings, meetings with your team mates (aka classmates), meetings with volunteers, etc.

2. Reflect on meeting details e.g. agenda items and what other important factors to take note of?

Responsibilities of Multi-Disciplinary Team (MDT)

- An identified manager and practice leader who oversee and facilitate the work of the MDT.
- A single process to access the workers in the team, with joint meetings to share insights and concerns.
- Electronic records of all contacts, assessments and interventions of team members with an individual and their family.
- A “key worker” system through which care for those with complex support packages are coordinated by a named team member.
- Some professionals work mainly within a single MDT in co-located premises. Others may be members of multiple MDTs and not co-located with other team members.

In the context of Active Ageing Centres (AACs), the Multi-Disciplinary Team (MDT) serves as a critical bridge between clinical healthcare and community-based social support. This collaborative framework ensures that seniors receive holistic, bio-psycho-social care rather than fragmented services.

Below is an elaboration of the MDT meeting structure, incorporating professional social service standards and the specific requirements of AAC settings.

1. Clarify Objectives

The primary goal is to shift from individual clinical interventions to an integrated care plan.

- **Holistic Goal Alignment:** Ensure all members (doctors, MSWs, OTs, etc.) are aligned on the senior’s “Ageing-in-Place” goals, prioritizing functional independence and social integration.
- **Risk Mitigation:** Identify high-risk seniors—such as those with high fall risks, cognitive decline, or severe social isolation—to prioritize immediate intervention.

- **Resource Optimization:** Align objectives with available community resources and AAC programs to prevent over-medicalization of social issues.

2. Review Rules and Dynamics

Effective MDT collaboration requires clear governance and a culture of mutual respect.

- **Leadership Roles:** Identify a **Practice Leader** or **Manager** to oversee the MDT's workflow and facilitate high-level decision-making.
- **Functional Roles:** Assign a **Facilitator** (to manage group dynamics), a **Recorder** (to update electronic health records), and a **Time-keeper**.
- **Inter-professional Ethics:** Establish ground rules for "Psychological Safety," allowing a junior AAC staff member to provide vital social context to a senior doctor or specialist without hesitation.

3. Review Agenda

The agenda should be structured to maximize the limited time of various healthcare specialists.

- **Case Prioritization:** Review the list of seniors categorized by complexity. Complex cases requiring "key worker" coordination take precedence.
- **Focus on Transitions:** Highlight seniors recently discharged from hospitals or those experiencing significant life changes (e.g., bereavement).
- **Time Allocation:** Ensure each profession (physiotherapy, nursing, social work) has a dedicated window to provide input on specific cases.

4. Work through Agenda Items

This is the core of the collaborative process, focusing on data-driven insights and joint assessments.

- **Integrated Case Discussion:** Review **Electronic Records** of all previous contacts, assessments, and interventions to ensure a "single source of truth" for the senior's history.
- **Joint Problem-Solving:** Share cross-disciplinary concerns—for example, a Physiotherapist noting a senior's physical decline while the AAC staff reports a lack of social attendance.
- **Social Prescribing:** Formulate interventions that combine medical care with social activities, such as befriending services or therapeutic group work offered at the AAC.

5. Plan Next Steps

Action items must be specific, assigned, and tracked to ensure accountability.

- **Key Worker Assignment:** For complex cases, name a specific team member as the **Key Worker** to coordinate the multi-pronged support package.
- **Intervention Tracking:** Schedule follow-up assessments and update the "Single Process" access point so all team members can track the progress of the intervention.
- **Next Meeting Objectives:** Define the "Review Cycle" (e.g., 3-month or 6-month review) for the cases discussed to evaluate the effectiveness of the care plan.

MDT Composition in AAC Settings

To ensure "sufficient diversity of professions", the MDT typically includes (but not restricted to):

Role	Primary Contribution to AAC
Doctor/Specialist	Clinical oversight and chronic disease management.
Nurse	Health monitoring and medication adherence.
PT / OT	Mobility assessments and home safety modifications.
Medical Social Worker	Navigating financial aid and family dynamics.
AAC Staff	Providing daily social observations and community engagement.

LU4: Devise strategic plan and framework in programme evaluation

Abilities (A4)

Plan strategic directions in programme evaluations and methods

3. Reflect and document in detail the use of a planning triangle to plan and evaluate your participated or initiated program or activity.



Utilizing the **Charities Evaluation Services (CES) Planning Triangle** allows the Multi-Disciplinary Team (MDT) to align service delivery with long-term quality-of-life improvements for seniors. By shifting from a purely clinical focus to an integrated bio-psycho-social model, the MDT ensures that "Ageing-in-Place" is a measurable reality rather than just a concept.

Below is the detailed evaluation framework for a representative program: **"Steady Steps: An Integrated Falls Prevention & Social Engagement Initiative."**

1. The Apex: Impact (Long-Term Change)

The ultimate goal of the MDT collaboration in an Active Ageing Centre (AAC) is to improve the overall effectiveness of the eldercare sector.

- **Definition:** The sustained benefit to the senior population over a period of 1–3 years.
- **Key Impact Areas:** * **Sustainable Ageing-in-Place:** Reducing the rate of premature nursing home admissions.

- **Enhanced Quality of Life (QoL):** Seniors maintain functional independence and a sense of purpose within their community.
- **Systemic Efficacy:** Improved integration between regional healthcare systems and community-based AACs.

2. The Middle: Outcomes and Indicators

Outcomes represent the specific changes in behavior, skills, or status that occur as a result of the MDT's interventions.

Outcomes (The "Changes")	Outcome Indicators (The "Evidence")
Increased Functional Independence: Seniors demonstrate improved physical ability to perform Activities of Daily Living (ADLs).	Number of seniors showing improved scores on the Timed Up and Go (TUG) or Berg Balance Scale .* Number of elderly successfully transferring from wheelchair to bed independently.
Enhanced Health Literacy & Self-Management: Seniors and caregivers gain the knowledge to manage chronic conditions and safety at home.	Number of seniors who can correctly identify their fall risks in the home post-intervention. Number of seniors adhering to medication regimes as verified by the MDT nurse.
Social Re-integration: Isolated seniors move from "at-risk" to "active" participants in the AAC community.	Decrease in scores on standardized Social Isolation Scales (e.g., Lubben Social Network Scale).

Prioritizing Outcome Indicators

In an MDT setting, the most critical indicators to evidence are those that track **functional independence** and **safety**. These are prioritized because they have the greatest direct impact on preventing hospital readmissions and maintaining the senior's dignity in their own home.

3. The Base: Outputs and Activities

Outputs are the tangible services delivered by the various MDT professionals.

- **Integrated MDT Activities:**
 - **Clinical & Rehab:** Physiotherapist-led strength and balance sessions; Nurse-led chronic disease screenings.
 - **Social & Environmental:** Occupational Therapist home safety

- assessments; Medical Social Worker (MSW) financial counseling for home modifications.
 - **Community Engagement:** AAC staff-led social interest groups and volunteer-led befriending.
 - **Output Indicators (Tracking the Work):**
 - Total number of physiotherapy or exercise sessions conducted.
 - Number of unique elderly residents supported by the MDT program.
 - Number of joint MDT case conferences held for complex "Key Worker" cases.
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MDT Role Integration in the Planning Triangle

To achieve these outcomes, the MDT must operate with a "single process" to access workers and share electronic records of all interventions.

- **Leadership:** An identified manager and practice leader oversee the program development to ensure the "diversity of professions" (Doctor, Nurse, PT, OT, MSW) is utilized effectively.
- **Coordination:** For seniors with the most complex needs, a "**Key Worker**" is assigned to coordinate the various outputs (e.g., syncing the PT's exercise plan with the MSW's transport subsidies) to ensure no gaps in the care continuum.